

Executive Summary: Abbott's FreeStyle Libre trial page offers patients a free sensor through the "Add to Wallet" feature, but it's unclear whether this engagement directly drives paid conversions. By running a controlled A/B test in Adobe Experience Manager and visualizing behavior with Contentsquare heatmaps, we can measure causation rather than correlation. Early modeling suggests a 3-point lift in conversion could add 7,500 paid users annually, representing \$9–\$22.5 million in incremental revenue. This low-cost, data-driven initiative strengthens customer loyalty, reduces churn, and positions Abbott to capture greater value in the competitive CGM market.

CASE STUDY: Abbott FreeStyle Libre's "Add to Wallet" and Patient Rx Lift

On Abbott's FreeStyle Libre trial page, users can click an *"Add to Wallet"* button to save a voucher for their free sensor. We've noticed that people who engage with this feature often show up later as paid customers. The correlation looks strong: more "Add to Wallet" clicks, more prescriptions filled. But causation isn't clear. Are these clicks actually driving conversions, or are motivated patients simply more likely to click *and* buy?

To answer that, we'd need to collect data from across functional teams and departments, including: website logs, voucher metadata, non-personal prescription data (ie, confirmation that the RX was valid and dispensed), conversion data showing user purchases beyond the free trial, and context or environment data (ie, regional campaigns, insurance coverage).

Testing Causation

The simplest way is an A/B test. Using Adobe Experience Manager (AEM), we can serve two versions of the trial page:

- **Control group:** current baseline "Add to Wallet" flow.
- **Treatment group:** optimized flow with clearer messaging, trust badges, and guided steps.

By randomizing users, we can measure whether the optimized experience actually lifts redemption and paid conversion. **Uplift modeling** helps isolate the incremental effect of the treatment, not just correlation with high-interest users. This approach aligns with Siegel’s (2016) emphasis on predictive analytics and the “ensemble effect,” where combining multiple signals improves accuracy and persuasiveness.

From a machine learning perspective, retrieval-augmented methods (Lewis et al., 2020) and sentiment analysis techniques (Chan, 2025) highlight how external behavioral signals can be layered into models to improve causal inference. In this case, combining clickstream data, voucher redemption, and prescription linkage provides a richer feature set for testing causality than any single stream alone.

Expected Outcomes (Illustrative)

Group	Add to Wallet Clicks	RX Redemption	Paid Conversion	Incremental Lift
Control	15%	12%	20%	—
Treatment	25%	18%	23%	+3%

If 250,000 free trials are issued annually, a 3 percentage point lift in paid conversion equates to **7,500 additional paid users**. At an average of \$1,200 per user annually, that’s **\$9 million in incremental revenue**.

Quantifying impact strengthens the case (our marketing managers may use different formulas):

- **Insured average cost:** Most commercially insured patients pay about \$70–\$99 per month for Libre sensors, or roughly **\$840–\$1,200 annually** (DeliverMyMeds, 2025).
 - $7,500 \times \$1,200 \approx$ **\$9 million incremental annual revenue**.
- **Cash price (no insurance):** Libre 3 sensors cost about \$235 for two sensors (28 days), equating to **\$3,000+ annually** (SingleCare, 2025).
 - $7,500 \times \$3,000 \approx$ **\$22.5 million incremental annual revenue**.

This range shows managers both the conservative insured impact and the upside potential if more patients pay cash or have higher out-of-pocket costs. Either way, optimizing the “Add to Wallet” flow has measurable financial value.

Beyond revenue, the strategic impact is reducing churn and increasing customer lifetime value. As Reichheld and Sasser (1990) argued in their classic “Zero Defections” study, retaining customers and preventing defections is often more valuable than new acquisition. By improving the trial-to-paid conversion journey, Abbott strengthens loyalty and reduces the risk of patients switching to competitors.

Managers don’t want to wade through model math. They need clear visuals which might include conversion funnel dashboard, lift chart, revenue impact table, confusion matrix heatmap (to show how well the model predicts converters vs. non-converters, highlighting precision and recall). We could also use heatmaps from Contentsquare showing where users click, scroll, and hesitate. Examples from Contentsquare’s site that show what these dashboards and heatmaps look like in practice: [Heatmap Capabilities Overview](#), [Real-World Heatmap](#) examples, and [Engagement Zone Heatmaps Guide](#).

Recommendation

We should move forward with a controlled A/B test of the “Add to Wallet” flow on the FreeStyle Libre trial page. AEM gives us the flexibility to serve optimized content and track engagement, and we can layer in Contentsquare heatmaps to visualize how users interact with each version of the page. That gives us both behavioral data and conversion metrics — the full picture.

If the optimized experience shows a measurable lift in paid conversion, we’ll have a clear business case to expand the changes across other channels (email, social, etc.). Marketing can use the insights to draft new content and deploy after clearing through the MLR process.

This approach is low-cost, fast to execute, and backed by real data. If successful, it directly supports Abbott’s strategic goals: increasing customer lifetime value, reducing churn (Reichheld & Sasser, 1990), and strengthening our competitive position in the CGM space. It also shows how modern machine learning approaches (Lewis et al., 2020; Chan, 2025) can be applied to real-world healthcare marketing challenges.

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